



Lorsedin[®]
ORAL SOLUTION

Composition :-

Each 5 mL (1 teaspoonful) contains Loratadine 5 mg

Appearance :-

Clear, colorless solution, orange flavor

Properties :-

Pharmacodynamics

Loratadine, the active ingredient is a tricyclic antihistamine with selective, peripheral H₁-receptor activity. Loratadine has no clinically significant sedative or anticholinergic properties in the majority of the population and when used at the recommended dosage. During long-term treatment there were no clinically significant changes in vital signs, laboratory test values, physical examinations or electrocardiograms. Loratadine has no significant H₂-receptor activity. It does not inhibit norepinephrine uptake and has practically no influence on cardiovascular function or on intrinsic cardiac pacemaker activity.

Pharmacokinetics

After oral administration, loratadine is rapidly and well absorbed and undergoes an extensive first pass metabolism, mainly by CYP3A4 and CYP2D6. The major metabolite-desloratadine (DL) is pharmacologically active and responsible for a large part of the clinical effect. Loratadine and DL achieve maximum plasma concentrations (T_{max}) between 1–1.5 hours and 1.5–3.7 hours after administration, respectively.

Increase in plasma concentrations of loratadine has been reported after concomitant use with ketoconazole, erythromycin, and cimetidine in controlled trials, but without clinically significant changes (including electrocardiographic).

Loratadine is highly bound (97 % to 99 %) and its active metabolite moderately bound (73 % to 76 %) to plasma proteins.

In healthy subjects, plasma distribution half-life of loratadine and its active metabolite are approximately 1 and 2 hours, respectively. The mean elimination half-life in healthy adult subjects were 8.4 hours (range = 3 to 20 hours) for loratadine and 28 hours (range = 8.8 to 92 hours) for the major active metabolite.

Approximately 40 % of the dose is excreted in the urine and 42 % in the faeces over a 10 day period and mainly in the form of conjugated metabolites.

Approximately 27 % of the dose is eliminated in the urine during the first 24 hours. Less than 1 % of the active substance is excreted unchanged in active form, as loratadine or DL.

The bioavailability parameters of loratadine and of the active metabolite are dose proportional. The pharmacokinetic profile of loratadine and its metabolites is comparable in healthy adult volunteers and in healthy geriatric volunteers.

Concomitant ingestion of food can delay slightly the absorption of loratadine but without influencing the clinical effect.

In patients with chronic renal impairment, both the AUC and peak plasma levels (C_{max}) increased for loratadine and its metabolite as compared to the AUCs and peak plasma levels (C_{max}) of patients with normal renal function. The mean elimination half-life of loratadine and its metabolite were not significantly different from that observed in normal subjects. Haemodialysis does not have an effect on the pharmacokinetics of loratadine or its active metabolite in subjects with chronic renal impairment.

In patients with chronic alcoholic liver disease, the AUC and peak plasma levels (C_{max}) of loratadine were double while the pharmacokinetic profile of the active metabolite was not significantly changed from that in patients with normal liver function. The elimination half-life for loratadine and its metabolite were 24 hours and 37 hours, respectively, and increased with increasing severity of liver disease. Loratadine and its active metabolite are excreted in the breast milk of lactating women.

Indications :-

Loratadine is indicated for the relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge (rhinorrhea) and itching, as well as ocular itching and burning. Nasal and ocular signs and symptoms are relieved rapidly after oral administration.

Loratadine is also indicated for relief of symptoms and signs of chronic urticaria and other allergic dermatologic disorders.

Dosage and administration :-

Adults and Children 12 years of age and over:

Loratadine Syrup: Two 5 ml spoonfuls (10 mg) once daily.

Children 6 to 12 years of age:

Loratadine Syrup:

Body Weight >30 kg - Two 5 ml spoonfuls (10 mg) syrup once daily.

Body Weight <30 kg - One 5 ml spoonful (5 mg) syrup once daily.

Children 2 to 6 years of age:

Loratadine Syrup:

Body Weight >30 kg - Two 5 ml spoonfuls (10 mg) syrup once daily.

Body Weight <30 kg - One 5 ml spoonful (5 mg) syrup once daily.

Safety and efficacy of Loratadine has not been established in children younger than 2 years of age. Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine; an initial dose of 5 mg or 5 ml once daily, or 10 mg or 10 ml every other day is recommended.

Pregnancy and lactation :-

Safe use of loratadine during pregnancy has not been established; therefore, use only if the potential benefit justifies the potential risk to fetus. Since loratadine is excreted in breast milk, a decision should be made whether to discontinue nursing or discontinue the drug.

Contraindication :-

Loratadine is contraindicated in patients who are hypersensitivity to the active substance or to any of the excipients in these formulations.

Warning and Precaution :-

Loratadine should be administered with caution in patients with severe liver impairment. The administration of Loratadine Products should be discontinued at least 48 hours before skin tests since antihistamines may prevent or reduce otherwise positive reactions to dermal reactivity index.

Warning :

Drugs known to inhibit hepatic metabolism should be coadministered with caution until definitive interaction studies can be completed. The number of subjects who concomitantly received macrolide antibiotics, ketoconazole, cimetidine, ranitidine, or theophylline along with loratadine in controlled clinical trials is too small to rule out possible drug interactions.

Effects on ability to drive and use machines

No impairment occurred in patients receiving loratadine. However, patients should be informed that very rarely some people experience drowsiness, which may affect their ability to drive or use machines.

Adverse drug reactions :-

In paediatric population, children aged 2 through 12 years, common adverse reactions reported in excess of placebo were headache, nervousness and fatigue.

The most frequent adverse reactions in adults and adolescents were somnolence, headache, increased appetite and insomnia. Other adverse reactions reported very rarely during the post-marketing period are listed below.

Immune system disorders

Anaphylaxis

Nervous system disorders

Dizziness

Cardiac disorders

Tachycardia, palpitation

Gastrointestinal disorders

Nausea, dry mouth, gastritis

Hepatobiliary disorders

Abnormal hepatic function

Skin and subcutaneous tissue disorders

Rash, alopecia

General disorders and administration site conditions

Fatigue

Drug interactions :-

When administered concomitantly with alcohol, loratadine has no potentiating effects as measured by psychomotor performance studies. Potential interaction may occur with all known inhibitors of CYP3A4 or CYP2D6 resulting in elevated levels of loratadine which may cause an increase in adverse events.

Symptoms and Treatment of Overdose :-

Overdosage with loratadine increase the occurrence of anticholinergic symptoms. Somnolence, tachycardia and headache have been reported with overdoses. In the event of overdose, general symptomatic and supportive measures are to be instituted and maintained for as long as necessary. Administration of activated charcoal as a slurry with water may be attempted. Gastric lavage may be considered. Loratadine is not removed by haemodialysis and it is not known if loratadine is removed by peritoneal dialysis. Medical monitoring of the patient is to be continued after emergency treatment.

Storage :-

Keep out of reach of children

Store below 30°C

Shelf-life :- Refer to unit box for shelf-life

Packages :-

Oral solution :- 60 mL bottle, contains Loratadine 5 mg/5 mL

Manufacturer :-

Siam Bheasach Co., Ltd.

123 Soi Chokechairuammitr, Vibhavadi-Rangsit Road, Chomphon, Chatuchak, Bangkok 10900 and 9 Soi Chokechairuammitr 3, Vibhavadi-Rangsit Road, Dindang, Dindang, Bangkok 10400, Thailand.

Product Registration Holder, Importer :-

SPG PHARMA (MALAYSIA) SDN BHD

Suite 1701, Block A, Damansara Intan e-Business Park, No 1, Jalan SS20/27, 47400 Petaling Jaya, Selangor Darul Ehsan, Malaysia.

LORS-5-01-L-ENG-MY-A0002

Copyright © 2022 Siam Bheasach. All rights reserved.

Actual Size 100 %

Drawing No. LORS-5-01-L-ENG-MY-A0002

Sent date 22/03/2023